

SECTION VII

Healing

CHAPTER 23

Healing Emotional Wounds

AN ATTEMPT AT RIGHTING THE WRONG

The parts that carry emotional hurt and physical pain from the past are often locked away deep within the psyche. It can sometimes take months or years to locate and gain access to these exiled parts, especially when you're dealing with more extreme forms of trauma and neglect. However, these parts tend to run the show, take a toll, and have a huge influence over a client's daily life. Many complex trauma systems spend much of their time and energy in the form of a *redo*, or an attempt to fix the problems of the past with someone in the present day. In other words, they end up in a continual state of redemption. For example, you may hear clients say things like: "My boyfriend will love me in a way that my father never did," "My friend Emily will be there for me in a way my mother never was," and "My kids will feel loved by me in a way that I never felt loved by my parents."

Exiled parts are in a perpetual state of wanting rescue. For many of them, fixing the past in the present day seems like the best available option. Unfortunately, however, this repeatedly fails and causes protective parts to once again jump in, which inevitably reinforces the original trauma. It's painful for me to see so many of my chronic trauma survivors stuck in a perpetual attempt at emancipation, which inevitably results in their picking a slightly upgraded version of their perpetrator and re-creating a version of their history.

In IFS, we offer a different kind of redo for our clients: a redo that involves their Self as the true corrective experience, not someone in their present day who shouldn't and can't help them right the wrong of long ago. The latter approach tends to have a very high failure rate. *The Self is the ultimate in corrective experiences for parts.*

Case in point: Consider an exchange I had with Darnell, a client with a strong and prominent caretaking part that, he learned, commandeered a large portion of his life and played a crucial part in his decision-making.

"When I ask inside, I hear that 20 percent of what I do for other people is related to wanting them to like me. I've known that about myself for a while now. But what I'm learning for the first time is that I also take care of people so they won't feel the same pain that my little boy did when he was abused, and that is a big percentage of this part. I'm hearing inside that this is about 80 percent," Darnell said. "This has

been a perpetual problem for me in my life in so many ways. I started a nonprofit organization, and this part of me has given away ridiculous amounts of money throughout my life. For Chrissake, I just paid for my daughter's best friend to go to private school next year. Do you think this part is also related to the trouble I've had with setting boundaries with people?"

"I don't know, Darnell," I replied. "Let's ask the part and see what it says."

"I know you keep harping on this boundary thing. But I'm now realizing that the reason it's so hard for me to say no to anyone is also related to this part trying its best to prevent people from feeling rejected like my little boy did back then," Darnell said. "I can see now how this has been an eternal problem for me that will never get fixed. Apparently, I keep trying to help the boy by taking care of everyone else. Thank goodness I'm finally getting it now. If we weren't doing this work, I think I would be bankrupt."

"Let the caretaking part know that we can release the little boy's pain, and it will no longer feel the need to take care of everything and everyone else all the time."

"He's nodding and smiling from ear to ear right now," Darnell said.

KEEPING IT HIDDEN

It's common and often uncomfortable for clients to exude the energy of a wounded, suffering victim. But it's important to help clients appreciate that this energy is an attempt by the exile to be seen, known, saved, and rescued without being consciously recognized or directly identified.

Therapists should work with the client on holding this *under-the-radar feeling* so the client knows it, acknowledges it, and keeps it alive without exposing too much of it in a way that activates protective parts. Therapists can gently name the energy of the exile and slowly bring it to conscious awareness even before protectors have given permission to formally access the wound. Saying too much too soon will be met with denial and pushback. But when the client displays enough Self-energy, naming what is below the surface can be helpful and illuminating to clients. I refer to this as *speaking beyond protectors* or implicit direct access with exiles. For example, consider talking to a client as such:

"Jody, I sense that there are some tender parts in there that are hurting. And I want them to know that we'll eventually come for them and help them let go of their sorrow," I said. "However, we need to do this in a way that's safe and respectful of all parts. When the timing is right, I promise we will come for them."

"When you just said that, I noticed that I instantly felt calmer," Jody said.

The awareness by the Self of the exiles' energy can help protectors and exiles alike feel comforted and hopeful.

It's also important to be mindful that with relational trauma, healing the exile may not automatically solve the problem. It's common for there to be multiple exiles that need to be cumulatively healed over time, and it's important to be aware that protective parts often carry their own pain and burdens that secondarily need attention.

THE ORDER OF HEALING

By trusting the wisdom of the Self, we let the client's system decide what order is appropriate when it comes to healing wounds. My clients will often want to make a list of their traumas to track their progress, which is fine with me. In fact, it can be quite useful, especially when their protective parts try to distract us by heading in other directions. Have your parts relax when it comes to what they think the order of healing should be. I have a general belief that clients move from the "outside in" when it comes to healing. In other words, they focus first on less intense trauma then move toward more painful and impactful trauma.

I was recently surprised when Kirsten, one of my clients, wanted to make a list of her traumas and proceeded to do so in what I would say was the typical order: less to more intense. As she started focusing on her relationship with her boss, she stopped herself mid-sentence and said:

"We need to talk about my mother. She's really at the root of all of my insecurities in life. If we focus on her, all the rest will naturally fall into place," Kirsten said.

"Sounds like a good plan," I noted.

I love this model and love trusting my clients' internal processes and healing wisdom. I would have never come up with that plan for Kirsten, and it was clearly the right way to go for her.

THE ISSUE OF RESPONSIBILITY AND ADDRESSING DISTORTED BELIEFS

It's common for parts of the system to show up that feel responsible for what happened to them, especially with trauma that occurs in childhood. Remember: If the one who was in power doesn't take responsibility for what they've done, it's natural for the recipient to have parts that believe it's their fault. Clients often say: "It must have been my fault" or "If I did or said something different, then

maybe it wouldn't have happened." When parts like this show up (and they frequently do), *I ideally have my client's Self challenge the distorted belief over my attempting to do it because it's often much more impactful*, as evidenced in the following exchange I had with a client:

"José, what do you want to say to that little boy who thinks it was his fault that his father hated him so much?"

Here, the Self is the great normalizer of the distortion, and when the connection between the part and the Self is securely in place, the part will likely be able to take it in.

"I want to tell him he was just a boy, and it was dad's responsibility to not get drunk and not yell at him all the time. It wasn't his fault at all," José said.

"Was he able to take that in?" I asked.

"Yes. His body softens as I say it to him."

A client of mine named Dawn had a horrific history of childhood trauma, which included a group of men sexually abusing her on multiple occasions. A particularly poignant session unfolded concerning this issue of responsibility when Self-energy was present. Dawn was able to listen to the little girl who lived her whole life feeling bad and believing that everything was her fault. Here is a sample of our conversation:

"Oh, my goodness—it feels like I'm finally getting it. The little girl inside of me is telling me that taking responsibility for everything was a way for her to not accept what those men did to her. She's saying, 'If I take responsibility for everything bad that happened to me, then I don't have to see what really did happen,'" Dawn said, now beginning to cry.

"It's a huge deal for her to let go of blaming herself and see and accept what really happened," Dawn continued. "She's also telling me that all the OCD stuff—the counting, the repeating, the lining everything up—is an extension of trying to take control and responsibility for every aspect of her life. It was another way in which she didn't have to see the reality. She's so exhausted from all of this. My heart is breaking for her right now."

"Great work, Dawn. I'm really glad you're fully getting this. Just stay with the little girl and give her what she needs right now," I said, feeling a full heart and high emotions because I was simultaneously connecting to the ways I tried to control and deny the reality of my own trauma.

Distorted beliefs held by parts within a traumatic system are often hidden, normalized, and out of conscious awareness. It's important for therapists to listen closely for these distortions, as they usually come up nonchalantly in therapeutic conversations. I was struck when a spiritual healer I

often see called out some beliefs that were indoctrinated into my system. “Frank, who says you can’t have a successful career and good relationships with your kids? What part of you thinks it needs to hold onto the pain of your childhood?” I was honestly shocked and taken aback when these beliefs were named. They felt like “truths” or “just the way it was.” I was blended with them, and they felt normal to me. Having these beliefs brought to my attention allowed me to connect with the parts that held them and enabled me to unburden the exile underneath that felt it wasn’t allowed and didn’t deserve to feel good.

DIFFERENT TYPES OF WOUNDS

In the section on attachment trauma, we talked about two different types of wounds that can develop in children when growing up. One is the *collective wound*, in which a main exile holds many different traumas over several years (e.g., “young Timmy” holding trauma due to the loss of his mother, the bullying he experienced at school because of his attention-deficit/hyperactivity disorder, and the fatal car accident he witnessed as a teen). The other wound type is what I call a *developmental wound*, where a similar trauma is experienced repetitively over several different developmental stages (e.g., a client’s 5-year-old, 8-year-old, and 12-year-old parts all hold aspects of dad being bipolar, intermittently out of control, and verbally abusive).

There is also what I call *parental wound pairing*, which occurs when children grow up in a dysfunctional family and develop a significant wound related to each parent (assuming it’s a two-parent household). Here, one wound is primary and the other is secondary, typically resulting from a part of one parent that is activated, hyperaroused, and in relationship with the other parent’s part that is withdrawn and hypoaroused (e.g., when one parent is verbally abusive and the other is passive, distant, and overly tolerant, or when one parent is sexually abusive and the other is dissociated and emotionally vacant). These parent pairings are unfortunately common, and children typically will develop two different wounds based on their responses to each of their parents’ parts. These are the clients who are typically diagnosed with borderline personality disorder or disorganized attachment in my experience.

Clients can also develop a set of parental pairing wounds that result from two activated parental parts or two blunted parental parts. These circumstances can be quite toxic for children and create deeply embedded wounds. When both parents are constantly fighting or are often emotionally vacant and unavailable, there’s no counterbalanced reality for children to experience. All they know is one side of an extreme, which can produce profound and intractable wounds internally.

Richard, a past client of mine, once described the difficulty he encountered setting limits with his oldest daughter. A portion of our dialogue follows:

“I know it sounds crazy. I know I should be able to do this, but I just can’t say no to her,” Richard said.

“Let’s see if you can go inside and ask if any parts know anything about this,” I suggested.

“Well, I see the little boy—the one who was the little husband to my mother. There’s still so much pain. He doesn’t want my daughter Sarah to ever feel anything close to what he’s feeling. If I disappoint her, she might feel sad or hurt like he did.”

“That makes sense, Richard.”

“I also remember constantly apologizing for everything I did when I was younger, even when I did nothing wrong. I remember thinking that if I was really good, maybe my mom wouldn’t do those things to me.”

“That makes sense too, Richard,” I said.

“But when I kept apologizing, my dad would always explode at me, especially when he was drinking. ‘Stop being such a goddamn baby,’ he would say over and over again.”

“I’m sorry you went through that as a child, Richard. It sounds horrible. Can you see if the little boy is holding both of these experiences—what your mom did to him and how your dad would explode?”

After a few moments, Richard said: “No. There are two separate kids in there. One is connected to my mom, and the other is related to my dad.”

“Okay, let them know we’re hearing them and that we’re going to help both of them get out of there for good,” I said. “Which one makes sense to focus on first?”

“Being touched and left alone is first, and being yelled at is second,” Richard said without hesitation.

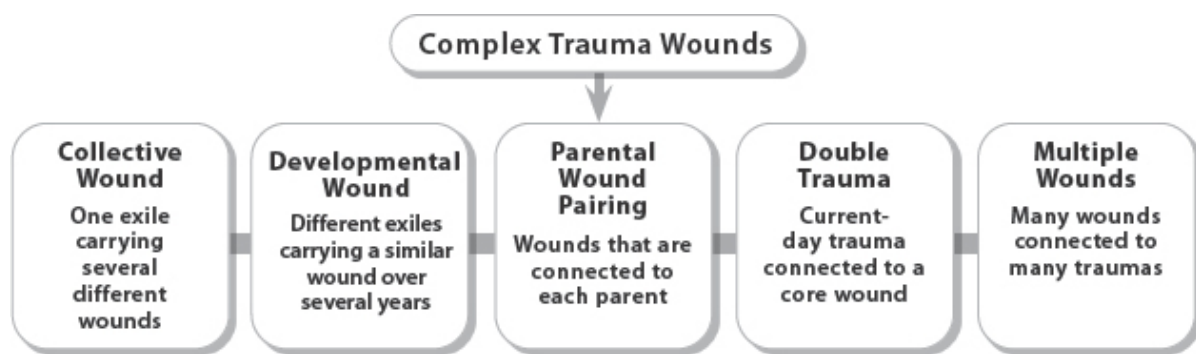
“Alright then, that’s how we’ll proceed.”

Most clients don’t come into treatment to heal their childhood wounds. Tom comes in because he’s drinking too much and it’s destroying his marriage. Juanita is underperforming at work and recently got fired from yet another job. I call these current-day traumas or present-day wounds, knowing that most of them are additionally connected to a core wound or past trauma. I refer to this trauma combination as a *double trauma*, one from the present connected to one from the past. For example, drinking too much affects Tom’s marriage, but it’s also protecting him from feeling unloved by his mother growing up. Or disconnecting and underperforming at work costs Juanita her job, but it also helps her avoid feeling invisible in her family as a child.

Most, if not all, current-day traumas have some link or connection to an earlier event or experience that's buried and was never fully dealt with. The present-day or secondary trauma compounds and adds to the pain of the primary or core wound. They both eventually need to be healed for protectors to let go of their job and for the system to feel fully free from the pain it shares or has in common. Many individuals experienced this during the COVID-19 pandemic, when the global trauma of quarantine, isolation, job loss, sickness, and death additionally triggered or reactivated difficult or painful experiences from their past.

Unfortunately, it's also possible for some trauma survivors to have *several wounds related to multiple traumas*. More often than not, but not exclusively, these clients will get a diagnosis of DID.

The following is a summary of the various types of wounds.



THE STEPS OF HEALING: THE UNBURDENING PROCESS

I always recommend that therapists who are interested in incorporating the unburdening process into their clinical practice complete Level 1 training through the IFS Institute (www.IFS-Institute.com). If you have not been fully trained, I suggest working with protective parts and—if and when an exile shows up—that you don't proceed with an unburdening but continue with what you're formally trained in or feel most comfortable with when dealing with vulnerability. If you are proceeding with IFS, once you and your client's Self have successfully encountered, connected with, and received permission from all protective parts associated with the wound in question, it's time for the client's present-day Self to meet the exile of the past. Here's an example, using a past session with a client named Ashley:

"How do you feel toward the girl inside who was abused by her older brother?" I asked.

"I'm feeling a lot of love for her right now," Ashley said.

"Let her know that, and see if she can take that in from you."

"She says she doesn't want sympathy."

Just as is the case with protective parts, exiles are often in connection with the Self of the past just before it left the overwhelming experience. We need to make sure they are fully updated and in connection with the Self of today before proceeding with the steps of unburdening.

“Ashley, see if she’s willing to see that you’re not that little girl anymore; see if she can see your essence now.”

“Yes, she’s seeing me now,” Ashley noted. “She says I’m really old.”

“Great. Give her some time to get to know and trust you.”

Now we’re ready for *witnessing*—the first step of the unburdening process, which allows the exiled part to share with the Self all that it’s carrying about the event. We make sure that the part shares not only the story but also the emotions, beliefs, and physical sensations associated with the trauma. Here is where *the exile often requires both empathy and compassion from the Self*. It needs the Self to be with it as a compassionate witness, and it often requires the Self to feel some of what it carries through empathic resonance. I often check in with the client’s Self periodically to make sure the amount of sharing isn’t overwhelming, as it can appear quite intense at times.

“Ashley, are you okay with what the little girl is sharing with you right now?” I asked.

“Yes,” she replied. “I’m getting for the first time the terror she experienced that day, and I want to be with her.”

“Great, let her know that.”

Once witnessing is complete, the part no longer feels like the only holder of the information around the trauma. For the first time, the part begins to feel that it no longer needs to carry the experience for the system; the Self can do that job. Be mindful that witnessing—or as Braca Ettinger, a French artist and psychoanalyst, refers to it: “wit(h)nessing” (Ettinger, 2001)—can take several sessions or even months to fully complete.

Do-over and *retrieval* come next. Here, we instruct the Self to enter the scene where the exile is and to give it what it needed and wanted but never got. First, the do-over allows for the corrective experience we covered earlier, where the Self is the corrective object for the part, not the therapist. This is often emotional for the therapist to witness and the client’s part to experience, as the Self is giving the part the love and attention it so desperately wanted and deserved. Next is the retrieval, where the Self takes the part out of the past and brings it somewhere safe in the present day. Here, I said to Ashley:

“Ashley, let that little girl know she will never have to go back there again.”

I want her to get the message that this will be permanent. I let the part choose where it wants to go as long as it's in the present day. I also want to make sure that the Self is with the little girl directly as opposed to a Self-like part showing up to help out.

"Ashley, are you seeing yourself with the little girl [a Self-like part], or are you there with her directly [the Self]?"

"I'm with her. She wants to go to my daughter's bedroom," Ashley replied.

"Great, let me know when she's there."

After the part is safe in the present day, we invite it to *unload* all of its thoughts, feelings, and physical sensations out of its body. This typically happens through a release of energy that's transferred to some form of nature, although this is not a requirement.

"She wants to first burn it in a fire and let it float up into the sky," Ashley said of her part.

"Good. Let her do that now, Ashley, and make sure that she releases all of it," I said, mindful that if the part does not fully unload all that it's carrying, the burden can return.

"She feels lighter and seems so much happier now."

"Terrific. Let her take in any qualities that she might need as she moves forward," I said, urging Ashley to take the step we call *the invitation*.

"She wants to take in confidence and have the ability to speak up."

"Good. Let her do that now."

The last step is *integration*.

"Ashley, let's bring in all the parts that have been protecting her and let them see her now."

"They're shocked," Ashley said. "They can't believe how happy, free, and relaxed she looks."

"Wonderful. Let them know they can let go of the job of protecting her now and that she's safe with you now. Also, let them know that we'll check in with them next week to see how they're doing and if they would like to unload anything they're still carrying."

This is often the case in trauma—that protective parts also need to unload what they were forced to hold in the service of protection.

The following diagram represents the six steps involved in the unburdening process.



DIFFERENT TYPES OF HEALING

One of the main things that makes healing from relational trauma so difficult is that there's often very little *access* to Self-energy in clients who have repeatedly been violated, betrayed, or attacked. This is a major frustration for therapists and clients alike. These clients have complicated, entrenched systems and experience difficulty convincing protective parts to relax and allow Self-energy to emerge. The container theory states that when the number and intensity of protective parts take up most of the space within "the container," there is less available space for the Self. I believe a combination of access and willingness to heal from a traumatized system allows the gift of Self-energy to emerge. It becomes an issue of exposing vulnerability for systems that have experienced the horrific.

Next, we'll explore what I believe are the different types of unburdening that occur when you're working with complex trauma survivors.

DIRECT ACCESS UNBURDENING

When protective parts hold more energy within a system than the Self does, a *direct access unburdening* is often necessary. Direct access unburdening is when the steps of the unburdening process are primarily performed by the therapist's Self in connection with the client's exile. This can feel extreme at times because the client is often fully blended in the wounded part's experience and

reliving the event to some degree. The therapist's Self does the witnessing and gives the part what it needed, wanted, and never got. (This step is particularly tricky, especially if the part wants to be held and the therapist does not routinely incorporate touch into their practice.) I generally discourage therapists from having physical contact with clients who have experienced abuse, as it can activate younger attachment desires and trigger the reexperience of abusive touch. You'll have to make your best clinical judgment if and when this occurs with each particular client.

The therapist then encourages the part to leave the past, to come into the present, to release what it's holding, and to enter new qualities, just as we do in a traditional unburdening. What's different is that the therapist is the Self here. This unburdening typically doesn't have the same impact on the client's system as does an unburdening that includes the client's Self. However, a direct access unburdening is often a necessary preliminary step in the overall healing process when a client doesn't have enough access to their Self. It ultimately helps to bring more Self-energy into the client's system.

This type of unburdening is rare and is usually needed during the early stages of treatment with clients who have DID. It is also beneficial with extreme protective parts that are unwilling to separate and that need to unburden what they're carrying before they are willing to give permission to access the wound. *I don't recommend trying this unless you feel very confident and comfortable with the IFS unburdening process*, as it can be overwhelming for even the most seasoned clinician.

One of my clients was very sweet when she explained the difference between direct access unburdening and the healing sessions we did when her Self was more present. She told me:

"Please don't take this the wrong way, Dr. Anderson. I appreciated it when you were the one helping my little girl, but it feels so different when I'm there too."

Anya, one of my clients, had an extremely anxious part that jumped in every time a feeling emerged. When I performed direct access with that protective part, we learned that it not only blocked all feelings, but it also carried them, including emotions strongly connected to her numerous childhood traumas. Remember, parts can hold a complicated range of feelings, beliefs, and behaviors. Anya's anxious part liked talking to me and enjoyed feeling heard in a way that it hadn't before. It was ready to give access to her 7-year-old girl who was sexually violated by her older brother.

"I'm not ready to include Anya in this yet," Anya's anxious part said. "I hold the feelings about missing our brother so much, and the 7-year-old holds what my brother did to us. Anya is acting too calm right now. I don't trust her."

"What if it were possible for you to be connected to both Anya and to the little girl at the same time?" I suggested to the anxious part.

“No. If I connect with Anya, I’ll lose the connection to the little girl. Anya still loves her brother too much.”

“I have a suggestion: What if the little girl shares her feeling with you and me first? Then, together, we can share your feelings and the little girl’s feelings with Anya. Would that be alright with you?” I asked the anxious part.

“Yes, that’s the only way it will work,” responded Anya’s anxious part.

“Okay, I trust that you know what’s best.”

“And I trust you,” the anxious part said. “Thanks for taking the time to listen to my side of things.”

“You’re very welcome,” I said.

Remember: Loving and appreciating the wisdom of parts and the natural order of healing is always the best way to move forward.

Anya and I then proceeded with a direct access unburdening. Her 7-year-old shared images of the event, while the anxious part shared feelings of grief and loss for her brother. It was incredibly moving to be the witness of the joining of these two parts. We later learned that Anya’s Self was watching this unfold from a distance.

“Finally, after all these years, I now know why I’m so uncomfortable with anyone touching me, why I won’t let anyone walk behind me, and why I cry like a baby at sad movies,” Anya remarked.

Clearly, our work had an important impact on Anya, her parts, and her therapist.

Some healings will begin as a direct access unburdening and evolve into a more traditional unburdening at the end. For example, witnessing will primarily be done by the therapist’s Self, and the client’s Self will later emerge to complete the remainder of the steps.

RELATIONAL HEALING

When there’s very little, if any, Self-energy available or accessible within your client’s system, direct access unburdening is often helpful, if not required. As the Self becomes more available over time, there’s a middle stage where the client’s Self is present but perhaps not present enough for the system to feel safe enough to attempt an unburdening without extra or outside help. This is what I call *relational unburdening*, where the client’s Self is present and the therapist’s Self or another person is needed to help complete the unburdening process. Relational unburdening is common with exiles that

hold attachment trauma, with exiles that have significant neglect histories, or with clients who carry a diagnosis of DID. *Remember, it's important to let the client's Self direct what the therapist's Self should do to help during the healing process.*

To illustrate this point, consider the following case involving a client named Luke, who had a relationally challenging adulthood. Before he entered therapy, he had been single for most of his life and desperately wanted to be in a relationship someday. We spent the first two years of therapy getting to know a range of his protectors, especially the harsh critic (internally and externally directed) and the highly intellectual part—which were both reluctant to step back and would constantly talk *about* what happened in Luke's life, always void of emotion. Eventually, we got permission from these pervasive parts to access some of Luke's exiles. Much to his surprise, we discovered a range of exiles that all carried similar experiences (i.e., a developmental wound).

“Now I understand why I never really knew who I was growing up. There are so many different parts in here,” Luke said.

“Let's just be open and try to listen to what they want to share with us. Let's leave it up to them, if that's okay with you,” I recommended.

“They seem to start around the age of 6 and go all the way up through my late teenage years—around 17 or 18,” Luke said. “They want to talk together in some ways but also have some specific things to share too. Is that okay?”

“It's totally fine with me.”

“They all seem so alone. They've been hiding out for years. The 17-year-old was the one who needed to step up to the plate and function in the world. The 12-year-old always felt so awkward in his body. And the 8-year-old never fit in socially at school. None of them could stand up for themselves, and no one ever felt good enough. The 6-year-old feels like crying right now, and so do I,” Luke said.

“It is sad, Luke.”

“One of the problems was that when they did cry, my mom would yell at them and say, ‘What's wrong with you? Stop crying!’”

“Luke, let them all know that we want to hear what they've been holding for you.”

“They feel like we're beginning to get it. They're holding so much sadness and feeling alone.”

“Luke, let them know that it's totally fine with us that they're sad. They have a lot to be sad about. Are you able to be with them right now in the way they need you to be?”

“No, not really, without your help. It's too much. They like and trust you a lot more than they trust me right now.”

“That’s alright. Let them know that you and I together can help them hold the sadness. I’ll be here every step of the way,” I assured Luke. “Let them share with us what they need to share.”

“It’s all about the feelings right now.”

“Luke, see if they can share the feelings a little bit at a time until they feel like we have all of it,” I suggested, trying to help them manage the overwhelm.

“Okay,” Luke said. A silent moment passed before he added, “I think we’re good.”

“Ask them just to make sure.”

“Yes, it’s alright now,” Luke said.

“Can you go into the scene and give them what they needed and wanted back then, and then bring them somewhere safe into the present?” I asked.

“Yes, they feel like they’ve gotten what they needed from us, and now they’re ready to leave.”

“Good. Do they want to go as a group, or does it feel better for them to go one at a time?” I asked.

“Definitely together.”

“Perfect, let them know they can unload all the feelings, thoughts, and beliefs they’ve been carrying in any way that works for them,” I suggested.

“It’s floating up into the sky and dissolving into thin air,” Luke described. “It’s so sad, but it also feels so peaceful.”

“Wonderful. When they’re ready, have them take in what they need to be able to move forward in life.”

“Love and belonging. And the assurance that they matter,” Luke said, with a calm and knowing look on his face. “Thank you so much, Frank. This was amazing.”

“I know. I feel honored to be sharing this with you right now, Luke.”

“This is the kind of closeness I’ve been missing all of my life.”

“Let them know you can have this in your life now, with someone who you love and want to share your life with,” I said with a pause. “Can we now check in with the critic and the intellectual part?”

“Oh, they’re both so happy. They’ve been watching this whole time. They are blown away. I feel so much love for all of these parts right now. It makes total sense to me now—they’ve been the watchdogs for years.”

“Keep sending them love and appreciation, Luke.”

“They love that. And they deserve it too,” a teary-eyed Luke said.

My presence was very important to Luke's parts, but I did let his system direct what needed to happen and in what order. I needed to be present for emotional and relational support without controlling the process or taking over.

Relational unburdening can also happen when working with a couple. Here, one member of the relationship is in the role of the observer while lending their Self-energy to the partner who is engaged in healing their wound (Herbine-Blank, Kerpelman, & Sweezy, 2016). This allows an unburdening to occur in the context of a loving connection, which can be quite powerful for both people in different ways. We know that Self-energy is a powerful healing force; collectively, it can add a level of safety and comfort to parts that carry and need to let go of their pain. We witness the collective power of Self-energy time and time again during live demonstrations that occur during IFS training.

CUMULATIVE UNBURDENING

Complex trauma survivors who engage in one-on-one therapy typically don't have access to the same collective Self-energy that is present in training demonstrations, so the healing process usually progresses at a slower pace. It takes a long time to get to know all the players in the system and for them to trust you enough to consider stepping back. Once a small percentage of the client's Self is available to connect with the wounded part, witnessing can happen a little bit during each session until the process is fully completed—which, in some cases, occurs over several months. With each session, a bit more witnessing takes place. This process is typically long and drawn out: Week after week, session after session, more of the story unfolds until the exile feels the Self and/or the therapist fully comprehends it. Witnessing this way, slowly over several weeks, is a great way for exiles to avoid overwhelming the system.

Once this is complete, the remaining steps of the unburdening process typically unfold over the next few sessions (i.e., the retrieval, do-over, unloading, invitation, and integration). I call this way of healing *cumulative unburdening*. This incremental healing transpires over several weeks with an extended witnessing phase. I also find that cumulative unburdenings don't necessarily follow the traditional steps in the typical order. In one session, there may be some witnessing and a bit of retrieval that occurs. In another follow-up session, there may be more witnessing and some unloading. Again, it happens gradually over multiple sessions.

Unburdening relational shame rooted in abandonment and neglect also takes much longer than a traditional unburdening. Protectors are fierce and reluctant to step back and give permission. Self-energy is often not readily available or minimally present, and if you move too quickly, parts push

back, and healing is less impactful for the system. This type of healing requires an enormous amount of patience and Self-energy from the therapist and can take weeks to months before it's complete due to the entrenched nature of the trauma.

For proof, ponder the case of Blake, a 46-year-old gay client who originally entered treatment because he was having difficulty in his relationship with his husband, Troy. It didn't take us long to learn about the origins of his intimacy problems. Blake had a strong internal storyteller who knew all the details of his abusive father, who left when Blake was 7 years old, and his alcoholic and critical mother who was left behind to raise four boys on her own. It took us over two years to get permission from all of Blake's parts to connect directly with the little boy inside. They were inflexible, often unwilling to soften, and reluctant to give us access, fearing the intensity of the boy's feelings. But eventually, Blake was able to connect with his Self, and his parts let him meet the boy, who ranged from 4 to 16 years old. Blake referred to this collective wound as "little B," a child who would be banished if he ever asked for anything—a boy who was constantly criticized by his "bitch of a mother," according to Blake.

"All you had to do was think about yourself," Blake said, tearing up. "Little B constantly felt like a nonentity. He had horrible self-esteem."

"Blake, are you hearing this directly from him, or is another part sharing this information?" I asked.

"It's coming from him. It's like he's transmitting his feelings directly to me. I'm really with him right now."

"Great. Let him know you're getting it, Blake."

"I remember when I got my first job in high school. I was afraid to even put my name on the badges we were required to wear. I didn't want anyone to know who I was or what a failure I had become. Sorry—I'll get back to little B," Blake said, admitting that he was alternating between different ages. "Little B and I are feeling so sad right now."

"It's totally fine. Let it unfold naturally," I instructed.

"He never thought anyone would ever stay with him."

"It sounds like he carries some shame, Blake."

"Oh my God, yes—an enormous amount."

"Let him know we'll keep coming back to him so he can share all that he's holding around your dad leaving and your mom being so nasty. Let him know we're here to help him."

"He says we have to do this a little at a time; that's all he can handle. He's saying there's a lot there, and it's going to take some time."

“Let him know that’s fine. We’re in no hurry here. He can take as long as he needs,” I replied.

“He really likes that.”

After about three months of slow witnessing, little B was ready to come into the present and unload what he was carrying. Blake’s whole demeanor changed: He became calmer, more introspective, less reactive, and more open to being vulnerable.

“After we did that work with little B,” Blake said, “my relationship with Troy transformed. I’m aware now that I was constantly treating Troy like the villain and that those feelings were mostly driven by little B. Thanks so much for helping me with this.”

Another example of a cumulative unburdening showed up with a client of mine named Janis who healed a school-aged little girl who felt unloved by her mother. I randomly checked in with Janis several months later to see how the girl was doing.

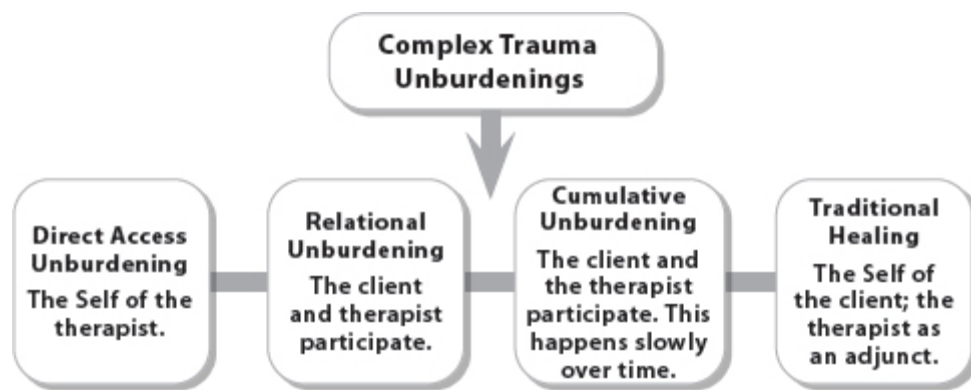
“To tell you the truth, I wasn’t able to be completely honest with her at that time because I was still drinking daily back then. I was as present as I could have been, but I think she needs another round of help now that I’m sober and can be more fully present for her,” Janis said.

Here, Janis was only capable of doing a small percentage of the healing due to her daily drinking. As she got sober, she became more capable of healing other layers of the little girl’s pain.

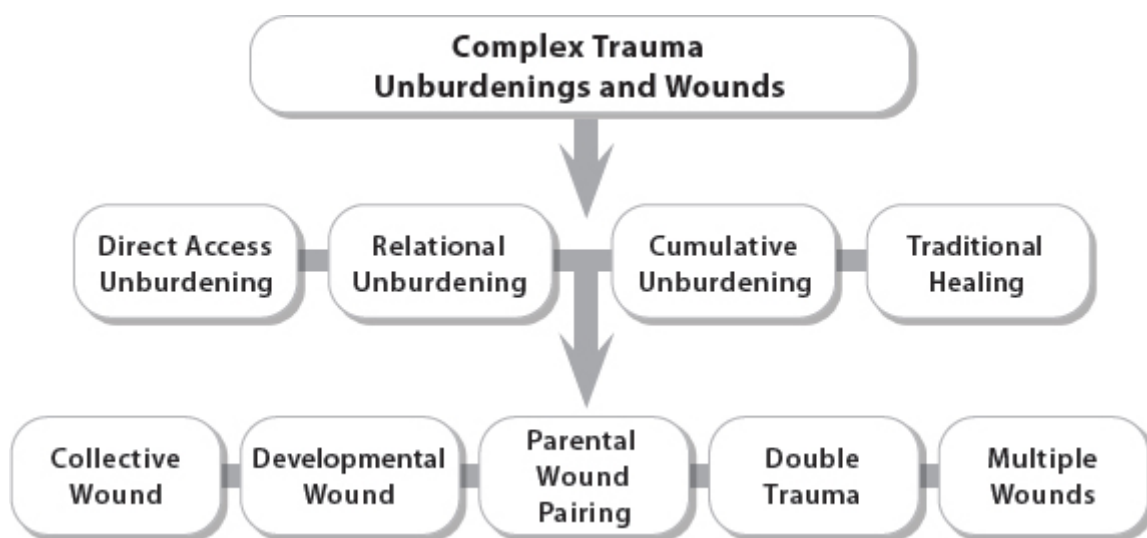
TRADITIONAL UNBURDENING

Traditional unburdening is the type of healing most of us are familiar with and are taught in IFS Level 1 training. This kind of unburdening can theoretically happen all in one sitting. It can be impactful, be powerful to witness, and typically have a big effect on the quality of a client’s life. Here, the client’s Self is the primary healing agent, and the therapist’s presence gently guides the process to completion. Often, this unburdening involves one exile carrying one wound. Or, with complex trauma, it can be one exile carrying several wounds, several exiles carrying a similar wound, exiles carrying a wound from each parent, exiles in the present connected to exiles in the past, or several different exiles carrying several different wounds.

The following graphic depicts the different types of unburdening that occur in relational trauma. They typically progress from left to right as treatment unfolds.



Here is a summary chart that represents the range of unburdenings along with the different configurations of wounds that show up in complex trauma.



Through an IFS lens, I believe there are various ways to heal traumatic wounds and several constellations of wounds that can occur in relational violations. There are numerous ways to unload or release pain outside of the IFS framework too. People can let go of the pain they carry in a variety of different ways: through religious experiences, corrective experiences, or healing relationships; by being in nature or reading something impactful; by using medicines; and via different psychotherapy modalities.

For me, IFS is the most comprehensive treatment modality. It incorporates thoughts, feelings, physical sensations, inner wisdom, energy, relationships, and spirituality. It makes the most intuitive sense to me, it fits my personality and relational style, and it has been transformational in healing my trauma history. I'm happy to say that I have healthy and fulfilling relationships with the people in my life who have been abusive to me, I have truly forgiven them, and I believe this would not have been possible without the healing capacity of IFS.

CHAPTER 24

Associated Features of Healing

THE POST-UNBURDENING PROCESS

The post-unburdening process is one I believe we don't talk enough about in IFS. Typically, during our training, we focus on, teach, and demonstrate the steps of healing and rarely have the opportunity to show or discuss what happens after an unburdening. Dick Schwartz says it can take about three to four weeks before an exile appears to integrate back into the system. Again, we don't exactly know what's happening in the brain during this process, but I sense that it has something to do with reinforcing the newly reorganized neural network. Hopefully, with further research, we'll have more clarity and definitive answers here. I recommend that clients *briefly check in with their unburdened part every day for about three to four weeks*, as Dick Schwartz has advised. I've noticed that when I've done this with my own newly healed parts, they gently fade into the background and become part of my overall system.

There seems to be an *integration phase* that occurs after healing. It's like putting the pieces of a puzzle together in a new, organic way, and this seems to happen both internally as well as externally. There's a *new way of being in the world*—one that no longer includes being triggered, which can take some getting used to for clients. There also seems to be a process of putting the past into perspective, seeing the world differently in the present, and allowing it to unfold without needing to be in control.

To demonstrate the power of the post-unburdening process, here is a sample of a session dialogue I shared with a client named Jena:

"Jena, how is your 36-year-old part doing this week?" I asked, referring to an unburdened adult part that was thrown to the floor by Mike (her husband) after he found out that she quit her job without telling him.

"I've been checking in with her like you had asked me to—not every day but most days," she said. "She feels stronger. She is more emotionally settled and feels like she's able to let go of that memory and the fear associated with it. She no longer feels unloved. She's more confident and doesn't need to constantly keep her distance from Mike anymore."

"That's great, Jena. I'm happy for her. How's your drinking part doing—the main part that was protecting her for all these years?"

“Hmm. He says he no longer needs to take care of her, but he’s not done drinking. He says there are other things in there he’s needing to watch over.” (As Jena demonstrates, parts can exhibit different genders.)

“Is he willing to share some of that with us now?” I asked.

“He’s still protecting the little girl who feels unloved, the teen who lost her brother to AIDS, and the woman who had to deal with a drunk husband for the past 25 years. He has a lot of fighting left to do.”

“Let him know that we’re grateful he’s willing to share all of this with us and that we’re committed to helping him if he wants us to. It seems like he’s protecting a lot of parts in there.”

“Yes. The little girl is ground zero, and all the other parts are layered on top of her. It feels like my whole life is beginning to make sense for the first time.”

After an unburdening has occurred, there’s *a new relationship formed between the Self and the former exiled part*. It can take time for the exile and protective parts to trust that things are different now and that the Self will consistently show up, no longer leave, and handle things differently. In some circumstances, there’s the potential for an overcorrection or backlash by protectors if the burden is not fully healed and protectors have not unloaded what they’re carrying yet.

Here’s a personal example of my healing a wound and how my parts were closely watching me as I navigated my world in a different way post-unburdening. I had unburdened a younger part of me that was frequently told what he felt and what he should feel. About a month later, my siblings and I were planning a surprise birthday party for my father, and they began suggesting to me what I should and shouldn’t feel. I started noticing some activation within myself, but I was surprisingly not reacting in my typical fashion. This time, I spoke up from the Self and not from my parts. I asked them if they were interested in hearing what “I” was feeling; they paused for a moment, apologized, and said: “Yes, of course we would.”

I knew there was no malintent from my siblings, and I was happy with how I was able to handle things differently after healing my wound. But what was even more surprising to me was how pleased my parts were that I was able to speak up. They watched me take the lead and were pleased by it. They saw that I didn’t get angry, withdraw, or just “take it”; I was instead able to stay in the conversation and represent myself differently. When I checked inside and saw my parts smiling at me, I understood the importance to them of my showing up differently. I felt a sense of warmth travel throughout my whole body.

It is important to check in with all parts after an unburdening, especially protective parts. They rarely spontaneously let go of their previous role within the system, particularly when it comes to relational trauma. They frequently continue their job and need to be *reintegrated* after a healing

session, which includes trusting the Self and letting go of their old role. Or they might need to *do an unburdening themselves*, based on how they feel about the job they were forced to do in the service of protection and preservation. You might hear a client's parts say: "I feel like a piece of garbage for the ways I've treated my wife when I've been drunk" or "I don't deserve anything good in my life after yelling so much at my kids and coworkers."

Remember: Extreme parts know the effect they've had on others, and they often carry enormous shame for their behavior. Take the time they need to be integrated and release what they're carrying. If care isn't given to them to help them let go of their former role and unload their guilty feelings, they can undo the unburdening and return the exile to the past.

Post-Unburdening Process

- **Check in daily with the unburdened part for three to four weeks.**
- **Adjust to a new way of being in the world.**
- **Secure the relationship between the Self and former exile.**
- **Integrate protectors by helping them trust the Self and let go of their old roles.**
- **Allow for protector unburdening if necessary.**

WHEN BURDENS RETURN

Certain circumstances can bring back a burden, even after it's been healed. The conditions under which this might occur are as follows: (1) the story was not fully witnessed by the Self, (2) there's no client follow-up after an unburdening, (3) something traumatic happens in the client's life soon after an unburdening occurred, (4) other parts within the system are using the burden for some other purpose, (5) protectors are threatened by the healing and bring the burden back, or (6) a legacy burden is connected to the wound (Schwartz & Sweezy, 2020).

I've also seen that clients with more severe histories, who only have a small percentage of the Self present during an unburdening, can reverse the healing if parts don't see the Self as being able to handle things consistently enough in the present.

I noticed, for instance, that my client Amanda was displaying some of her old passive behaviors again. So I asked her to check in and see how her teenage part—the one we had helped six months earlier heal from a rape—was doing. After doing so, Amanda said, much to her surprise:

“I guess the girl has gone back into the past.”

After further inquiry, we learned that the teenager had seen Amanda get depressed and had noticed that she wasn’t standing up for herself anymore. The girl no longer felt safe and had lost faith in Amanda’s Self as her depression worsened and her protective parts became more prominent.

This can be a common scenario, especially with clients who struggle with DID and don’t have enough Self-energy consistently showing up in their lives. Protective parts or even exiles can decide to reverse the healing and return into the past.

The following is a list of the common reasons why a burden might return.

- **The story is not fully witnessed.**
- **There’s no client follow-up after the unburdening.**
- **Something traumatic happens in the client’s life.**
- **Other parts within the system are using the burden.**
- **When protectors are threatened and bring the burden back.**
- **A legacy burden is connected to the wound.**
- **There’s not enough Self-energy present for parts to trust.**

TRAUMA MEMORIES

We are therapists, not truth seekers. It’s not our job to determine whether a trauma did or didn’t occur. And, yes, it’s common for clients to have parts that doubt whether or not it happened; it’s also common for clients to have parts that ask our opinion. I try to steer clear of joining either side of that polarization. *Our job as IFS therapists is to help clients help their parts unload whatever they’re carrying around a particular event in their lives.* We’re not there to determine whether or not it happened as they remembered it. We stay with their experience without judgment or opinion and help them release the feelings, beliefs, and physical sensations around it. The *perception* of parts can have a powerful impact on their lives, and we can help them with that.

Anyone who works with couples knows the impact of perception. Often, one partner will have an entirely different view of what happened during an argument than the other partner, and each of their perspectives will have a huge impact on the relationship.

Again, we cannot and should not confirm or reject the accuracy of the trauma memories without definitive proof. But we can help parts unload whatever feelings they carry around it without adding to, denying, or altering their autobiographical memory.

BACKLASH

Protector backlash can be harsh, brutal, and even violent at times, especially when extreme protector parts haven't given full permission to access or heal the wounds they protect. It's one of the reasons we take getting permission so seriously in IFS. Therapists can get frustrated when it is difficult to get permission and might take advantage of the moment when an exile "storms the gate" and breaks through the protective barrier in a desperate attempt to get help.

Many of us were also psychodynamically trained and always taught to "go for the affect when it shows up." However, if an exile breaks through and you proceed with an unburdening without full permission from protectors, the healing can be reversed; even worse, severe protectors can harm clients as a punishment for going to places they've not approved of. According to Dick Schwartz, some reasons for backlash include loyalty to the perpetrator, lack of protector follow-up after an unburdening, and parts' concern over feeling content or happy.

This last point—that feeling good is not always a good thing—is something that has baffled me for a long time. I've had many clients who have courageously sorted through difficult experiences over several sessions, and we've just accomplished a big piece of work together. I've secretly expected them to arrive at the next therapy session and profusely thank me for all the ways I've aided them. Instead, they've shown up and proceeded to tell me how bad things are or how horrible they have felt since our last session. My heart has dropped with frustration and disappointment.

I've now grown accustomed to clients who suffer from relational trauma and routinely have parts of them that feel unsafe or uncomfortable when things go well and they experience joy or happiness. I've learned that the good is unfamiliar and often uncomfortable for these parts and that it often means bad things are just around the corner for them.

I now appreciate these parts. I'm committed to gently helping them acclimate to a new normal, knowing that it will take them time to adjust.

FORGIVENESS, REPAIR, AND THE POWER OF HEALING

Forgiveness and repair can be tricky for many trauma survivors. Some clients say they have no interest in repairing the relationship with their perpetrator, and they permanently sever all ties. Others

will stay connected in the relationship with their abuser in an ongoing way that appears to be unhealthy. And some will forgive prematurely and struggle with not getting what they had hoped for as a response.

My suggestion to all of my clients is this: *Heal the wounds first, and then see how you feel about reaching out to your perpetrator.* Once clients release the wounds that another has caused, they are freed up, they no longer carry the trauma inside themselves, and they are untethered from their perpetrator. They possess a power from within that's rooted in love and compassion. They often have a different perspective about their feelings toward the offender and what they want to say to them.

At times, all that's required is for parts to feel truly seen and heard by the Self, and no contact with the perpetrator is necessary. At other times, after the wound is healed, clients need to name it (in a non-attacking way) and to speak up for what they know to be true. The process of reclaiming power through Self-led confrontation is impactful both internally and externally. And at other times, they're able to identify the exile underneath the perpetrator and, with a feeling of compassion, can let go and forgive. Whatever clients decide to do about confrontation, forgiveness, or repair, I always feel honored to bear witness to their process, and I trust they'll do what's best for them moving forward.

THE SCIENCE BEHIND THE HEALING

I believe that memory reconsolidation, a form of transformational neuroplasticity that Bruce Ecker talks about in his book *Unlocking the Emotional Brain*, nicely dovetails with what happens during the unburdening process in IFS. First, Ecker describes what's called the *accessing sequence*, where one identifies a current symptom and brings it from implicit (unconscious) to explicit (conscious) awareness (Ecker et al., 2012). We achieve this in IFS when protective parts grant the Self permission to be with buried or suppressed memories held within the exiled or hidden parts of our client.

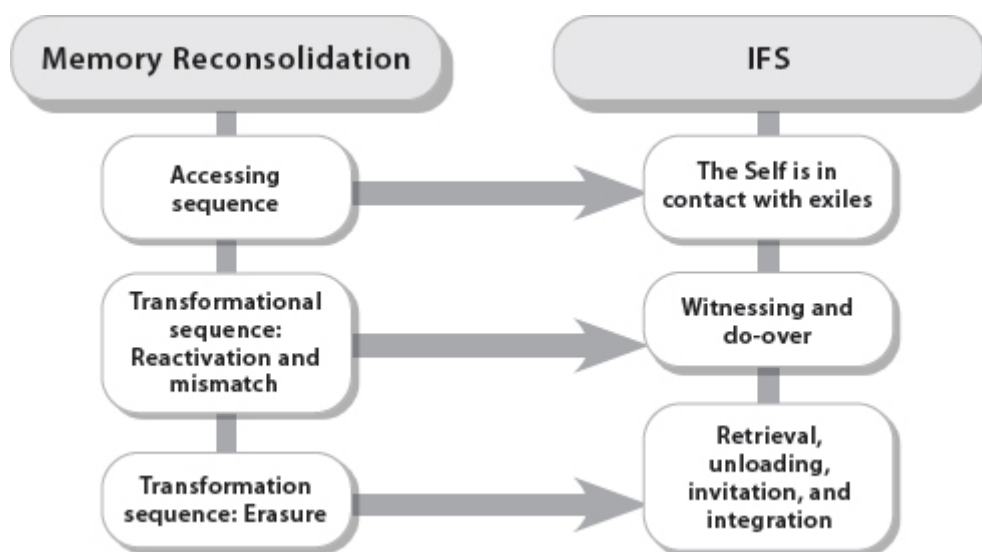
Ecker next talks about the *transformation sequence*. Here, one *reactivates* the target symptom and simultaneously couples it with a *mismatch* or full contradiction of the original target's experience. The reactivation allows for the synapses on the neural network to become temporarily unstable, setting the stage for the mismatch, which promotes the correcting, updating, and rewiring of the neural network in question (Ecker et al., 2012). We see this in IFS during the witnessing and do-over steps of the unburdening process, when the Self can be with, listen to, validate, and give the part what it needed, wanted, and never got. In other words, the Self provides the exile with an opposite and fully corrective experience from its prior experience.

The next aspect of the transformation sequence includes *erasure* or revising the emotional memory with new learnings, which typically happens within approximately a five-hour window of

time (Ecker et al., 2012). We see this during the retrieval, unloading, invitation, and integration steps. Here, exiles are invited to enter the present day; release their thoughts, feelings, and physical sensations; and take in the new qualities they need to move forward. We also bring protectors back so they can relinquish their old roles. All of these steps are new experiences for exiles and protectors alike.

Ecker claims that memory reconsolidation is the only known form of neuroplasticity that's capable of truly eradicating emotional learning by unlocking synapses in existing emotional memory, which doesn't impair autobiographical memory. He also compares this to counteractive change, which competes with unwanted learnings by building up new preferred learnings that override and suppress the existing ones; an example of a counteractive change would be CBT (Ecker et al., 2012).

The following chart shows the association between the accessing and transformation sequences in memory reconsolidation with the IFS unbundling process steps involved.



CHAPTER 25

Roadblocks to Healing

OVERCOMING THE BARRIERS

Workshops and online courses that focus on treating trauma survivors are consistently some of the most popular offerings for therapists because helping clients overcome their traumatic experiences is one of the most challenging aspects of our profession. Again, extreme circumstances create extreme reactions, which creates clients who are challenging for therapists to treat.

Some common barriers consistently show up during the treatment of complex PTSD, but with proper awareness and the presence of Self-energy, you can overcome them and help your clients heal, have better connections, and access more love. This is one of the motivations for me to write this book. I have aimed to collate and share many of the experiences I've acquired over my 29 years of working with trauma survivors in the hopes that it will help you more seamlessly navigate the bumpy waters of trauma treatment with less distress and more success.

Next, we'll explore some of the common challenges IFS therapists frequently encounter during the trauma healing journey.

Psychotherapy can be moving along at a reasonable pace. Then suddenly, things start slowing down, protectors seem more entrenched, and access to Self-energy appears to wane. Here's where I look to see if an *underlying biological process* might have taken hold of my client. Is there a newly developed depression? Are they suddenly more reactive or dissociative? Did the client start drinking after a long stint of sobriety? Any of these scenarios can be a sign of an emerging biological condition in your client. It can feel like a sudden regression in the therapy, a slow-moving fog, or a heavy blanket that envelops the Self and parts. If you identify and treat this swiftly, the therapy will naturally resume; if not, sessions can continue to dredge along at a snail's pace and cause a great deal of frustration for you and your client. Remember to always ask parts if they feel that something biological is affecting them. They're usually accurate.

Some clients present with what I call *a crisis-driven system*: a series of horrible events that consistently show up with no apparent solution. These parts are more often hyperaroused in presentation and seem to thrive in chaos. I try to not get too caught up in the drama; I see these systems as largely protective, aware that the ongoing drama keeps the feelings and vulnerability

away. Don't chase the crisis, as it fuels the avoidance. Over the years, I've learned to be laid back and nonreactive to these constant traumas. At times, it might even appear to my clients that I'm not very caring. But I've learned to tolerate a lot in the service of showing extreme protectors that I can handle their intensity while I continually offer them a solution to the drama: healing the wound.

We all can't be Dick Schwartz. After all, many of us have not been developing and practicing IFS for the last 35 years like he has. I'm often in awe of his extraordinary ability to put protectors at ease and to unburden exiles. But you don't have to have Dick's years of experience to be a good IFS therapist. *Know your limitations*; trauma treatment is a specialized form of therapy, and I believe that IFS is perfectly suited to help clients effectively overcome their adversity. Most importantly, *continue to work on yourself and heal your trauma*. The more unburdening I've done over the years, the less my parts show up in the office and the more I live my life guided by Self-energy. I am convinced this is precisely why the IFS model is spreading so rapidly throughout the world: It's effective, and it changes lives.

When a *therapist's parts* emerge during a client's session, boundary issues commonly arise. Most of the consultations I do are related to therapist parts. They either have parts show up that are controlling and too rigid, protecting against their own feelings of being intruded upon, or they have parts that are too soft and often cross the therapeutic line, giving to others what their internal system wanted and never got. Both extremes are common therapist parts that impede progress. Once healed, they no longer feel the need to show up in therapy.

A common and easy-to-remedy roadblock is the *lack of follow-through from session to session*. "Hello. What would you like to talk about today?" is something I usually say to identify a target part at the beginning of treatment. After that, I rarely say it again. Once we've contracted to help a target part, I want to make sure that we honor that commitment. I know that parts are watching me to see if I can be trusted and follow through with what I say. And I also know that different parts tend to show up each week to distract us from identifying and healing pain. Picking up right where you left off during the previous session will dramatically increase your chances of gaining protector permission and unburdening exiles.

Again, extreme circumstances warrant extreme responses. Many complex trauma survivors have unyielding systems that are incredibly effective at keeping the sorrow away. As a result, therapists can often feel frustrated, discouraged, and hopeless with the slow pace of progress in trauma therapy. I've found it hard at times to consistently hold the IFS frame and to stay the course with some of my more chronic clients. When things get difficult and the therapy feels stuck, it's easy to abandon the key step of trying to get permission from a resolute protector, and it's common to forget that your

client has a Self, which can seem next to impossible to access sometimes. Also keep in mind that some extreme trauma protectors carry so much pain that they *need to be unburdened themselves* before they're willing to give permission to access the wound. This is a frequently overlooked roadblock but can be easily remedied by directly asking the part if it needs to unload what it's carrying before or after the exile.

It's customary for those new to the IFS model to give up on IFS and to resort back to their old therapeutic techniques due to sheer frustration. I call this the *drift factor*: When the going gets tough and progress feels like an unrealistic pipe dream, we can lose confidence in our ability to help clients heal using IFS. As a result, we return to easier and more familiar methods, including traditional talk therapy. I know that I've certainly done this before when I can't seem to make progress in accessing my client's Self or in adequately addressing a staunch protector's fear. It's like taking a commercial break: I resort to a mostly talk therapy session and then try IFS again the next session.

Another common roadblock discussed earlier is *getting indoctrinated into the client's protective belief system*. They can wear us down over time, and we begin to believe them when they say things like: "I am much better off living alone" or "I'm too damaged to ever fully heal from the assault." These unwavering parts can be steadfast in their position, and we can gradually start to believe them. I suspect this subtle brainwashing happens more often than we're consciously aware of. This has happened to me on more than one occasion, especially with some of my longer-term chronic clients. This is why having an IFS supervision group can be enormously beneficial: having an outside perspective can help you through difficult, stuck phases in your treatment with clients.

LEGACY BURDENS

It's impossible to live in the world as we know it without carrying some form of a legacy burden. These are burdens that result from the the family we grew up in, the culture we live in, our race, our gender, our ethnicity, our sexual orientation, and other factors. *Legacy burdens often block trauma healing*, and therapists frequently don't pay attention to their presence or the impact they can have on their clients.

Part of the reason it's easy to overlook legacy burdens is that people tend to normalize their presence after living with them most of their life. It's what they've always known, and these beliefs become integrated into how they see themselves and who they think they are in the world. The good news here is that when therapists point them out, clients are usually able to identify them and release them in a variety of different ways. Dick Schwartz, Deran Young (founder of Black Therapists Rock), and Ann Sinko (a lead trainer for the IFS Institute) are among the experts in our community who

speak about legacy burdens. Ann has written a chapter about legacy burdens in the book *Innovations and Elaborations in Internal Family Systems Therapy* (Sinko, 2017).

When unloading a legacy burden, it's important to help clients differentiate what percentage of the burden belongs to them (i.e., has become an internalized part of them) and what portion isn't theirs (i.e., has entered their system but belongs to someone or something outside of themselves). In either case, the legacy burden can be released. It's common for clients to be reluctant to release legacy burdens that arise within the family due to loyalty issues connected to certain relatives, especially those who are abusive. Self-energy is a great normalizer here, so addressing any parts that block access to the Self is an important step. The Self helps clients differentiate between right and wrong and gain clarity around what's important to hold on to and what they can let go of. Legacy burdens can be unloaded without the need to be witnessed, but sometimes they can carry important messages that they need to share with the client before they're willing to leave the system. It's important to help clients hear the message and realize they don't need to carry the burden's energy.

Here's a portion of a conversation I had with a client named Danielle to illustrate this point:

"As my parts step back, I see myself as a skeleton," Danielle said. "I have consistently given myself away to the men I've been in a relationship with most of my life. It started with my first boyfriend, and now I realize I've done it with every man I've been with since—including my husband."

Danielle continued: "I see myself with my mother; we're holding hands. I'm like her. We're connected in this way, giving ourselves away. This is who we are and what we do."

"Danielle, are you interested in no longer carrying this burden you share with your mother?" I asked.

"Most definitely. It's been a heavy weight for a long time now, for both of us."

Another client of mine, Ryan, was struggling with how hard it was for him to connect with people and with his feelings. Upon further inquiry, he also became aware that it had been challenging for him to feel empathy. Ryan's partner wondered if he was "on the spectrum," but I assured him that he wasn't. He had spent a great deal of time "figuring out" how to relate to people.

"If you look them straight in the eyes and repeat back to them what they say to you, their faces light up and they usually respond favorably," Ryan said. "It's very effective."

Ryan's response sounded calculated and intellectual. I saw for a moment why his partner thought he might be on the spectrum. However, after further exploration, Ryan was surprised to discover a transgenerational burden the men in his family carried.

“None of the men on my dad’s side of the family could show vulnerability. This included my grandfather, my dad, me, and, unfortunately, my son and grandson too. The message was always, first and foremost, serve others and don’t express yourself. Be strong, be stable, and help your fellow man. I come from a long line of preachers, and feelings weren’t part of the trade; helping others was the main priority.”

“Believe it or not, this is something that you don’t have to carry anymore,” I told him. “There’s a way to release this belief if you want to, Ryan.”

“That would be incredible,” he responded.

Legacy burdens are ubiquitous; they often come from the people we grow up around, the organizations we associate with, and the culture in which we are raised. They can be subtle or dramatic in presentation. And they’re often an integral part of our internal environment and typically need to be released before traumatic wounds can heal.

THE SCIENCE BEHIND LEGACY BURDENS

The IFS model has been attending to and releasing legacy burdens for quite some time, and it’s exciting to finally connect them with scientific knowledge that describes the transgenerational transmission of trauma through epigenetic mechanisms.

A paper by Rachel Yehuda, one of the main researchers looking at the epigenetic transmission of PTSD, summarized much of the findings in the field and stated that there are two broad categories in offspring that can be affected by underlying epigenetic mechanisms. The first is accommodations made by offspring in response to environmental exposure early on in life; the second is the effect of preconception trauma in both parents that gets genetically transmitted to offspring through both egg and sperm. These effects can happen in utero, as well as postnatally or after birth (Yehuda & Lehrner, 2018).

More simply stated, *kids can exhibit symptoms of trauma as a result of the environment in which they grow up, as well as the genes they inherit from their parents*. A child can actually display PTSD symptoms without ever experiencing a traumatic event. DNA methylation is a proposed mechanism that is responsible for modifying gene expression and influencing the rate of genetic mutation in response to environmental challenges (Stenz et al., 2018).

Scientists believe that a better understanding of the transmission and development of PTSD will lead to future interventions and pharmacologic treatments that can reverse the consequences of early life stressors and promote greater resilience in children (Stenz et al., 2018). IFS has found a way to

release these environmental and genetic effects of trauma by helping clients unload the burdens they carry from ancestors and heal the wounds they hold within their parts.

UNATTACHED BURDENS

The last roadblock to healing we'll examine is called an *unattached burden*. This is when negative energy from the environment attaches to your client's system. This is something you may not agree with or believe exists, and that is totally fine. I'm not here to impose any of my beliefs on you but to merely present what I've encountered in my practice. In my experience, people who have endured significant trauma often dissociate, or frequently leave their bodies, and can have negative energies attached to their system. These negative energies tend to block Self-energy and make it harder for clients to unburden their traumatic wounds. At first glance, unattached burdens can appear as scary, menacing, unsophisticated, and unidimensional, but their bark is much worse than their bite. They can attach to a particular part of a client, or they can attach to the client's system as a whole.

The way to determine if an unattached burden is present is to simply ask if the "part" belongs to the client or if it came from somewhere else. Believe it or not, the part will usually answer this question honestly. The process of releasing an unattached burden is similar to releasing a legacy burden; however, therapists must ensure that they, as well as their clients, embody as much Self-energy as possible because love, light, and the Self seem to be the antidote to these unattached burdens. At times, we'll even ask our clients to call in additional support from whatever help source they trust, be it God, Buddha, Jesus, Allah, or even a family member that has passed.

It's important to be brave in the face of unattached burdens, as they tend to feed off fear. As Dick often says in reference to unattached burdens: "Nothing can hurt you *internally* if you're not afraid of it." Our goal is to help disperse these energies from our client's system and to transform them into something positive. However, this requires an advanced skill that we typically teach in IFS, Trauma, and Neuroscience Level 2 training and in Level 3 training at the IFS Institute.

To comprehend the process involved with releasing unattached burdens, explore the following exchange I had with a client named David:

"The swirling energy is back again," David said. "Every time I try to get close to the 10-year-old boy, it comes in and shuts everything down."

"Can you focus on it for a few seconds and see if you can determine whether it's one of your parts doing this or if it comes from somewhere else?" I asked.

"That's weird. It's not my part," David said definitively. "How do you know that?"

“I didn’t know it for sure. But in my experience, if we ask they’ll tell us.”

“I’m seeing this for the first time. I took on their energy,” David said.

“Whose energy?”

“It’s a combination of my mom’s and dad’s energy. Her desperate sadness and the way she never let anyone get close to her. And his intense, depressed, and sexually inappropriate demeanor,” David explained. “It feels like evil energy that’s inside both of them.”

“We can release it, David,” I said. “It sounds like it doesn’t belong to you and that it might not belong to your parents either. Can you check that out?”

“I’m getting an affirmative, that it doesn’t belong to them or come from their families. This is so bizarre.”

“I understand that this feels a bit strange. But it’s just energy, and we can release it in a way similar to how we’ve been able to release other painful things in your life. It’s not that different,” I said, trying to normalize the experience for him.

“Let’s do it then. I don’t want it, and I don’t want them to have it either.”

“I understand. We can send it away and transform it into something positive. David, bring your parents up in your mind’s eye. As you focus on the swirling energy, I’m going to ask any parts of you that might be afraid of it to temporarily go into a waiting room.”

“Okay, they’re in the waiting room.”

“Great. Now, this may sound strange, but let’s have you, me, your mom, and your dad send the swirling energy as much love and light as we can. This is the best way we can help it to transform.”

“I get that,” David replied. After a few moments, he said: “We’re all together, and there’s this huge light; it’s like a beacon or an enormous flashlight, and it’s just shining down upon all of us. The swirling energy is slowly dissolving.”

“That’s beautiful, David. Just let it continue to do that until the energy is completely gone.”

“This is amazing. We’re all so peaceful together.”

Releasing legacy-burden energy, perpetrator energy, or unattached-burden energy is similar in practice and theory. It’s crucial not to react negatively toward it or to be afraid of it. Instead, stay calm, steady, and within Self-energy as you and your client send love, light, and compassion to help it transform. Asking for outside help from God, guides, angels, or ancestors sometimes helps but isn’t necessary. Occasionally, we ask the energy if it has any wisdom it wants to impart before it leaves.

The following is a list of the common roadblocks associated with the treatment of complex PTSD survivors.

- **An underlying biological condition**
- **A crisis-driven system**
- **Therapist limitations and unhealed trauma**
- **Therapist parts get activated**
- **Lack of follow-through from session to session**
- **Extreme parts need to be unburdened first**
- **Hard to stay on course (the drift factor)**
- **Getting indoctrinated into the client's belief system**
- **Legacy burdens**
- **Unattached burdens**

CHAPTER 26

Spiritual Dimensions of Healing

GETTING OUTSIDE HELP

It's important to be mindful that our personal spiritual beliefs may or may not align with those of our clients. We need to be able and willing to move these beliefs aside if necessary, so we can be fully open and available to our clients. For example, I've had clients spontaneously mention that angels, ancestors who have passed, spirit animals, and Mother Mary appear during an unburdening session. This is often surprising to them.

When clients receive information from beyond, it's important to help them work with any skeptical parts that may come forward. Ask them: "What is the quality of the information? Does it feel more like a thought, or does it seem to come from outside themselves? Is it peaceful or disturbing?" These are questions I often ask my clients to help them differentiate between a part of them and spiritual information from beyond them, which generally has a peaceful quality and calms the system.

For instance, examine the following discourse between me and a client named Brin:

"Dr. Anderson, this is freaking me out. I'm having this weird feeling that my dog is going to die within the next few months. I'm not sure why this keeps coming up. I frequently think about it during the day at work, and sometimes I even dream about it at night. Is this some sort of premonition, or am I batshit crazy?" Brin asked.

"I'm not sure where this information is coming from either," I told her. "But in my experience, there's a qualitative difference between our thoughts, which can sometimes feel intrusive and are often rooted in fear, versus information that is sent to us from someplace outside ourselves. When it's the latter, it tends to feel soothing and has a helpful quality associated with it. Are you able to differentiate between these two regarding this information?"

"Yes," Brin replied, without hesitation. "I know what you're talking about. It's coming from beyond me. I know it, but I tend to ignore this stuff all the time. I know my dog Sammy is getting old, and I've seen signs of her failing for a while now. I can't bear the idea of losing her, and I sense that this outside force is trying to prepare me for her passing."

"Can you slow things down for a moment and just thank whoever is trying to help?"

“Yes. It feels good to have someone watching out for me like this,” Brin said. “Stuff like this used to happen to my grandmother all the time too.”

“I’m glad you’re open to receiving help, Brin.”

BEYOND THE SELF

Arjun, a client of mine who teaches high school math, was struggling with multiple medical problems, including a recent stroke, joint pain, and several environmental allergies that were triggering asthma attacks. He complained during one of our sessions that he felt like his body was falling apart and that he no longer had any control over his life. Control was something that Arjun struggled with for a long time, and he began to know it as a protective part.

“I was the only one who was capable and competent at home growing up,” Arjun told me. “My mom was always popping one pill or another. And my dad’s depression often got the best of him, making him an ineffective father and provider most of the time. It was my job, even as a kid, to be the competent one—the one in charge at home.”

Arjun continued: “You keep telling me to go inside and listen to what my body’s trying to tell me about all of this. But I’m having a hell of a time doing that. I feel like my body is totally out of control right now.”

“Are you open to trying a little experiment, Arjun?” I asked. “What if you tried to let go for a moment and see if you could be open to trusting what’s beyond you? What if you asked for help, trusting the goodness that’s all around you, and observed what happens? Do you remember what it was like when your parts were able to take in and feel your love and support? What if you were able to try this outside of yourself?” I asked.

“I get it, Frank. My little guy liked feeling my love and support, but letting go of control and being open to what’s beyond me is tough to do. Old habits die hard.”

“I know, Arjun. But I wonder if you’d be open to trusting it just a little bit, right now in this moment?”

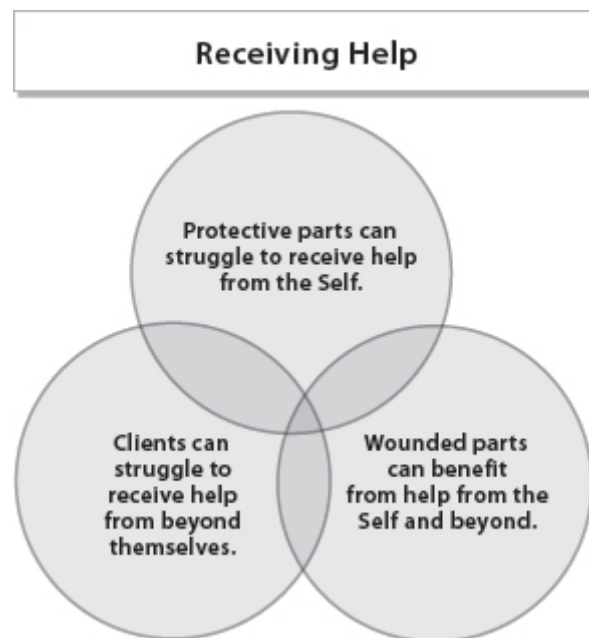
“I’m scared, but I’ll try it.”

After a moment of silence, I felt the energy shift in the room.

Arjun said: “It’s funny, but I feel calmer right now. Something so simple has been so hard for me to do. Thank you.”

The suggestion to trust, let go, and ask for help is notoriously difficult for most trauma survivors, especially when control was taken away from them at an early age. Letting go and asking for support is often challenging, but it is also relieving to parts that needed to be in control for so long.

Even though getting help from outside the Self is common and often essential in the treatment of trauma, very little can substitute a genuine and authentic Self-to-part relationship. Help from God, guides, angels, a partner, or even a therapist can be useful and incredibly impactful. I have personally found that one layer of healing often occurs with the help from outside support and, later, another layer happens between my Self and parts. Having access to what's within and what's beyond is the best of both worlds and can have a huge impact on healing the distress caused by trauma.



PERSONAL VIEW ON TRAUMA, LOVE, AND CONNECTION

I believe we are born into this world to have a richly rewarding human experience, learn life lessons, and experience love, creativity, and connection. I also believe that we all encounter various degrees of trauma and hardship throughout our lives that disconnect us from our Self. When we view trauma as our teacher, we address it, heal it, learn from it, and undergo growth and transformation at the soul level. I can personally say that my trauma has been a gift to me. The pain and adversity I've gone through, processed, and healed from has shaped me, taught me, helped me grow, and provided me with a rewarding career. It has helped me write this book, which hopefully will benefit countless others in overcoming their difficult life experiences.

Some of our lessons may be the result of interactions we have with people in our current life, and I believe some may show up through past life experiences. We all have been given free will and, therefore, have a choice as to how we handle our adversities. We can take advantage of them, we can bury them, or we can relive them in an attempt to repair them. I believe that adversity is an

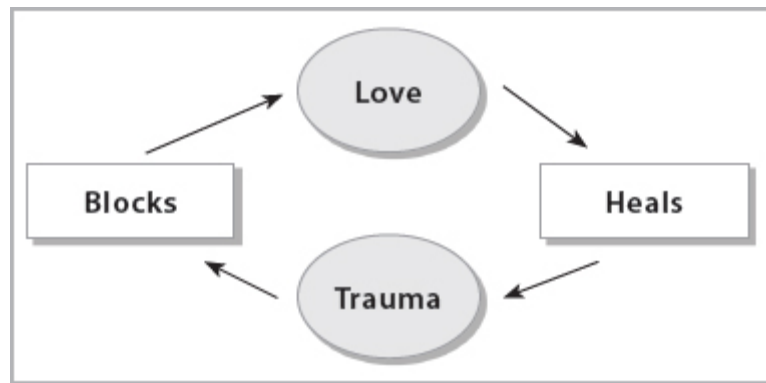
opportunity to learn and grow, and when we choose to do so, it allows us to become more aligned with our truth and more connected to our source, which I trust is within us, around us, and infused with love and goodness.

I also believe that when we're connected to our core and aligned with our source, we can tap into the love and creative energy that's readily available to us. Creativity is an essential life force, which I believe is at the intersection between our soul and our source. When you're in a creative flow, it feels effortless, expansive, and magical, and you're able to produce wondrous things. We frequently see and experience this when we personally connect with various forms of art, such as music, movies, paintings, plays, and photography. Creativity can move us, make us feel something, and touch us deeply on an individual as well as a global level. It can change the world.

I feel that we're connected to an all-knowing and loving presence within us, which IFS refers to as Self-energy. I further maintain that when we're born, we simultaneously experience a separation and loss from this source energy. We live life with a deep yearning and desire to connect with that divine energy; therefore, we seek out love and connection here on Earth through our various relationships. I am convinced that our desire for connection is an attempt to unite with the love that's inherently in each of us and ultimately in the divine. In other words, when we love and connect with another person, we are connecting with God.

I further believe that the soul shares a connection with many different lifetimes, and each time we are reborn, we've contracted to learn something new and experience a different dimension of life. For example, one life experience could involve being the victim of something horrific, another could be living a privileged life, one could involve being a perpetrator, and another could be living the life of a caretaker. To truly know what it's like to be human, we need to walk in all shoes and to fully experience all aspects of life, not from a place of hate and judgment but from one of love and acceptance.

Whether our trauma or life lessons originate from the past or the present, it blocks our connection to the Self and the love embodied by the divine. I truly believe and have personally experienced that all forms and dimensions of trauma can be healed. When that happens, we learn something valuable about life. Our soul grows. And we gain greater access to the love, light, and creativity that is within us and all around us.



Trauma blocks love, connection, and creativity; and in turn, love has the power to transcend trauma, heal us, and reconnect us to our Self, our soul, our source, and each other.

Thank you for joining me on this journey.

Love, Frank